Confirm your alectinib treatment setting This guide covers alectinib alone for ALK-positive lung cancer. Confirm whether it is adjuvant treatment after complete surgery, first treatment for advanced cancer, or treatment after crizotinib. These settings have different evidence and treatment lengths.

NCCP 00401 · VERSION 6

Alectinib monotherapy

Take alectinib twice daily with food, every day, with no planned rest week. After surgery, treatment lasts up to 24 months, stopping sooner for recurrence or unacceptable side effects. For advanced cancer, treatment continues until progression or unacceptable side effects under the oncology plan.

Discussion aid: review with the patient and complete the approved local consent process.

Purpose and plan

☐ Exact indication, treatment aim and uncertainty explained

☐ Expected duration and review / switch points discussed

☐ Alternatives and the option not to proceed discussed

☐ Patient priorities, capacity and support needs explored

Practical safety

☐ Medicine, route, schedule and missed-dose instructions

☐ Food, interactions and safe handling reviewed

☐ Pregnancy, contraception / fertility and breastfeeding

☐ Monitoring, symptom reporting and contact plan

Selected material risks

☐ Muscle pain or weakness and raised CPK

☐ Anaemia, including destruction of red blood cells (haemolysis)

☐ Lung inflammation (ILD/pneumonitis), which can be serious

☐ Skin rash and sensitivity to sunlight

☐ Potential harm to a developing baby

☐ Liver injury and raised liver tests or bilirubin

☐ Slow heart rate, dizziness or fainting

☐ Constipation, nausea, diarrhoea and swelling

☐ Gastrointestinal perforation; urgent severe abdominal pain

Discuss the individual aim, the evidence for your setting, alternatives including not proceeding, uncertainty, material risks, pregnancy/fertility, daily capsules, monitoring and access. You can ask for an interpreter, more time or another explanation and can reconsider. This aid supports a consent conversation; it does not replace it.

Individual benefit, alternatives and concerns

Agreed checks and next review

Liver profile every 2 weeks for the first 3 months, then every 4 weeks or as clinically indicated. FBC and renal profile every 4 weeks. CPK every 2 weeks for the first month, then as clinically indicated, including muscle symptoms. Blood pressure and pulse as clinically indicated. NCCP says ECG prior to cycle 2, then as indicated; a continuous oral regimen has no defined cycle length, so agree the first repeat ECG appointment locally. Assess symptoms, interactions, adherence, current dose and treatment benefit; scans and disease monitoring follow the Consultant plan.

Clinician / date: _____ Patient discussion documented: _____

Consent is an ongoing conversation and may be reconsidered. This aid is not treatment authorisation.